

Diagnostic criteria for Amphetamine or Similarly Acting Sympathomimetic Intoxication

- A. Recent use of amphetamine or similarly acting sympathomimetic.
- B. Within one hour of use, at least two of the following psychological symptoms:
 - (1) psychomotor agitation
 - (2) elation
 - (3) grandiosity
 - (4) loquacity
 - (5) hypervigilance
- C. Within one hour of use, at least two of the following physical symptoms:
 - (1) tachycardia
 - (2) pupillary dilation
 - (3) elevated blood pressure
 - (4) perspiration or chills
 - (5) nausea or vomiting
- D. Maladaptive behavioral effects, e.g., fighting, impaired judgment, interference with social or occupational functioning.
- E. Not due to any other physical or mental disorder.

292.81 Amphetamine or Similarly Acting Sympathomimetic Delirium

The essential feature is a Delirium (p. 104) within 24 hours of intake of an amphetamine or similarly acting sympathomimetic.

Associated features. Tactile and olfactory hallucinations may be present. Affect is often labile. Violent or aggressive behavior is common, and restraint may be required.

Course. Delirium usually occurs within one hour of substance use and is over in about six hours. When the substance is taken intravenously, the onset is almost immediate. More rarely the Delirium follows a period of intoxication. When the other pharmacological effects of the substance have worn off, the Delirium disappears completely.

Complications. See Cocaine Intoxication, p. 146.

Prevalence. This disorder is not as common as Amphetamine or Similarly Acting Sympathomimetic Delusional Disorder.

Differential diagnosis. See differential diagnosis of Delirium, p. 106.

Diagnostic criteria for Amphetamine or Similarly Acting Sympathomimetic Delirium

- A. Delirium (p. 107) within 24 hours of use of amphetamine or similarly acting sympathomimetic.
- B. Not due to any other physical or mental disorder.

292.11 Amphetamine or Similarly Acting Sympathomimetic Delusional Disorder

The essential feature is an Organic Delusional Syndrome (p. 114) following recent use of an amphetamine or similarly acting sympathomimetic during a period of long-term use of moderate or high doses of the substance. (The syndrome apparently does not develop following a single large dose unless preceded by chronic use.) The syndrome develops rapidly. Persecutory delusions are the predominant clinical feature; in addition, there are ideas of reference, aggressiveness and hostility, anxiety, or psychomotor agitation.

Associated features. Distortion of body image and misperception of people's faces may occur. Initially, suspiciousness and curiosity may be experienced with pleasure. However, suspiciousness and paranoid delusions may later induce aggressive or violent action against "enemies." The hallucination of bugs or vermin crawling in or under the skin (formication) can lead to scratching and extensive skin excoriations.

Course. Delusions can linger for a week or more, but occasionally last for over a year.

Impairment, complications, and differential diagnosis. See Organic Delusional Syndrome, p. 114.

Diagnostic criteria for Amphetamine or Similarly Acting Sympathomimetic Delusional Disorder

- A. Recent use of amphetamine or similarly acting sympathomimetic during a period of long-term use of moderate or high doses.
- B. A rapidly developing syndrome consisting of persecutory delusions as the predominant clinical feature and at least three of the following:
 - (1) ideas of reference
 - (2) aggressiveness and hostility

- (3) anxiety
- (4) psychomotor agitation

C. Not due to any other physical or mental disorder.

292.00 Amphetamine or Similarly Acting Sympathomimetic Withdrawal

The essential feature is a characteristic withdrawal syndrome due to recent cessation of or reduction in use of amphetamine or a similarly acting sympathomimetic. The syndrome always involves depressed mood, plus fatigue, disturbed sleep, or increased dreaming.

Associated features. If the depressed mood is severe, other symptoms of the depressive syndrome, such as agitation or suicidal ideation, may be present. The disturbed sleep is usually related to increased REM sleep activity, and may last for weeks.

Course. The syndrome develops within three days of cessation of or reduction in substance use. The symptoms reach a peak at two to four days, although depression and irritability may persist for months.

Complications. A Depressive Disorder and suicide are the major complications.

Differential diagnosis. A coexisting **Depressive Disorder** should be considered if a depressive syndrome persists for several weeks.

Diagnostic criteria for Amphetamine or Similarly Acting Sympathomimetic Withdrawal

A. Prolonged heavy use of amphetamine or a similarly acting sympathomimetic.

B. After cessation of or reduction in substance use, depressed mood and at least two of the following:

- (1) fatigue
- (2) disturbed sleep
- (3) increased dreaming

C. Not due to any other physical or mental disorder, such as Amphetamine or Similarly Acting Sympathomimetic Delusional Disorder.

PHENCYCLIDINE (PCP) OR SIMILARLY ACTING ARYLCYCLOHEXYLAMINE ORGANIC MENTAL DISORDERS

This group of substances includes phencyclidine and similarly acting compounds such as ketamine (Ketalar) and the thiophene analogue of phencyclidine (TCP).

These substances can be taken orally or parenterally or can be smoked or inhaled. Within this class of substances, phencyclidine is the most commonly used. It is sold on the street under a variety of names, the most common of which are PCP, PeaCe Pill, angel dust, THC, and crystal.

305.90 Phencyclidine or Similarly Acting Arylcyclohexylamine Intoxication

The essential features are specific physical and psychological symptoms associated with maladaptive behavioral effects due to the recent use of phencyclidine or a similarly acting arylcyclohexylamine. The symptoms begin within one hour of oral ingestion of the substance; if smoked, insufflated, or taken intravenously, onset may be within 5 minutes. The physical symptoms include vertical and horizontal nystagmus, elevated blood pressure, numbness or diminished responsiveness to pain, ataxia, dysarthria, and diaphoresis or increased salivation. Psychological symptoms include euphoria, psychomotor agitation, anxiety, emotional lability, grandiosity, a sensation of slowed time, and synesthesias (e.g., seeing colors when a loud sound occurs).

The effects of this class of substance are generally dose related, although there is great variability among individuals. The effects usually range from a mild, "floaty" euphoria and numbness after ingesting less than 5 mg of phencyclidine, to muscle rigidity, hypertension, and a noncommunicative state following a dose of 5-10 mg, and coma, convulsions, and possible death after a dose of 20 mg or more of phencyclidine.

Associated features. Intoxication may be accompanied by repetitive motor movements, including facial grimacing, muscle rigidity on stimulation, and repeated episodes of vomiting. There may also be hallucinations, paranoid ideation, and bizarre or violent behavior.

Course. In most cases, individuals who are acutely confused following ingestion of phencyclidine or a similarly acting arylcyclohexylamine are alert and oriented within 3-4 hours of admission to an emergency room. Chronic users of phencyclidine report feeling intoxicated for 4-6 hours after ingesting the usual "street" dose.

Complications. Death from respiratory depression can occur following a high dose. Suicide is not uncommon during acute intoxication. While an individual is recovering from an intoxicated state, depression, irritability, and nervousness often occur.

Differential diagnosis. Other Substance-induced Intoxications that cause a similar clinical picture, such as those due to amphetamines and hallucinogens, may be ruled out by the presence of phencyclidine in the urine or plasma.

Diagnostic criteria for Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Intoxication

A. Recent use of phencyclidine or a similarly acting arylcyclohexylamine.

B. Within an hour (less when smoked, insufflated, or used intravenously), at least two of the following physical symptoms:

- (1) vertical or horizontal nystagmus
- (2) increased blood pressure and heart rate
- (3) numbness or diminished responsiveness to pain
- (4) ataxia
- (5) dysarthria

C. Within one hour, at least two of the following psychological symptoms:

- (1) euphoria
- (2) psychomotor agitation
- (3) marked anxiety
- (4) emotional lability
- (5) grandiosity
- (6) sensation of slowed time
- (7) synesthesias

D. Maladaptive behavioral effects, e.g., belligerence, impulsivity, unpredictability, impaired judgment, assaultiveness.

E. Not due to any other physical or mental disorder, e.g., Delirium.

292.81 Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Delirium

The essential feature is a Delirium (p. 104) due to phencyclidine (PCP) or a similarly acting arylcyclohexylamine. The Delirium may occur within 24 hours after use or may emerge following recovery from an overdose days after the substance has been taken.

Associated features. See Phencyclidine Intoxication, p. 151.

Course. The Delirium may last up to a week, with waxing and waning that is probably a reflection of excretion into and reabsorption from the stomach.

Complications. See Phencyclidine Intoxication, p. 151.

Prevalence. This disorder is not as common as Intoxication from this class of substances.

Differential diagnosis. See differential diagnosis of Delirium, p. 106.

Diagnostic criteria for Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Delirium

- A. Delirium (p. 107) due to phencyclidine (PCP) or similarly acting arylcyclohexylamine.
- B. Not due to any other physical or mental disorder.

292.90 Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Mixed Organic Mental Disorder

The essential feature is recent use of phencyclidine or a similarly acting arylcyclohexylamine that has resulted in an illness that involves features of several Organic Brain Syndromes or a progression from one Organic Brain Syndrome to another. For example, an individual may simultaneously have prominent delusions, hallucinations, and signs of disorientation, or may initially have a Delirium, followed by an Organic Delusional Syndrome.

Diagnostic criteria for Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Mixed Organic Mental Disorder

- A. There is evidence of recent use of phencyclidine or a similarly acting arylcyclohexylamine.
- B. The resulting illness involves features of several Organic Brain Syndromes or a progression from one Organic Brain Syndrome to another, e.g., initially there is Delirium, followed by an Organic Delusional Syndrome.

HALLUCINOGEN ORGANIC MENTAL DISORDERS

This group includes two types of substances, both of which have hallucinogenic properties: substances structurally related to 5-hydroxytryptamine (e.g., lysergic acid diethylamine [LSD] and dimethyltryptamine [DMT]), and substances related to catecholamine (e.g., mescaline).^{*} These substances are taken orally.

305.30 Hallucinogen Hallucinosi

The essential features are characteristic perceptual changes, physical symptoms, and maladaptive behavioral effects due to recent hallucinogen ingestion. The perceptual changes include subjective intensification of perceptions, depersonalization, derealization, illusions, hallucinations, or synesthesias (e.g., seeing colors when a loud sound occurs). These occur in a state of full wakefulness and alertness. There may be hyperacusis and overattention to detail. The illusions may

^{*} Phencyclidine (PCP), although it has been referred to as an hallucinogen, is classified separately (p. 150) since it rarely causes a pure hallucinosis.

involve distortions of the individual's body image. The hallucinations are usually visual, often of geometric forms and figures, sometimes of persons and objects. More rarely, auditory and tactile hallucinations are experienced.

Physical symptoms include pupillary dilation, tachycardia, sweating, palpitations, blurring of vision, tremors, and incoordination.

The maladaptive behavioral effects may take the form of marked anxiety or depression, fear of losing one's mind, ideas of reference, paranoid ideation, impaired judgment, interference with social or occupational functioning, or failure to meet responsibilities.

This category is called an Hallucinosis even though it is recognized that frequently, with low doses, the perceptual changes do not include hallucinations.

Associated features. The associated features are heavily influenced by the setting in which the syndrome occurs, the dose, and the expectations and personality of the individual. Euphoria is common. Usually the individual realizes that the perceptual changes are due to the effect of the hallucinogen. More rarely, the individual is convinced that he or she has lost his or her sanity and will not regard it.

Imagery and thoughts are often dominated by mystical or religious experiences. For example, the individual may believe that he or she has achieved certain insights not possible otherwise.

Course. The onset is usually within an hour of ingestion. In the case of LSD, the most commonly used hallucinogen, the disorder lasts about 6 hours. For other hallucinogens the duration may range from under an hour to a day or two, at most, three days.

Complications. In rare cases the individual will act irrationally and may harm himself or herself or others. "Flashback" hallucinations (recurrent hallucinations after the hallucinogen is no longer present in the body) can occur. Hallucinogen Delusional Disorder and Hallucinogen Affective Disorder are possible complications.

Differential diagnosis. See Organic Hallucinosis, p. 116. Various **Substance-induced Intoxications**, such as that due to **cannabis**, cause clinical pictures similar to the Hallucinogen Hallucinosis that results from low doses. In such cases, if an adequate history is not available, laboratory tests should be used to make the diagnosis.

Diagnostic criteria for Hallucinogen Hallucinosis

A. Recent ingestion of a hallucinogen.

B. Perceptual changes occurring in a state of full wakefulness and alertness, e.g., subjective intensification of perceptions, depersonalization, derealization, illusions, hallucinations, synesthesias.

C. At least two of the following physical symptoms:

- (1) pupillary dilation
- (2) tachycardia
- (3) sweating
- (4) palpitations
- (5) blurring of vision
- (6) tremors
- (7) incoordination

D. Maladaptive behavioral effects, e.g., marked anxiety or depression, ideas of reference, fear of losing one's mind, paranoid ideation, impaired judgment, interference with social or occupational functioning.

E. Not due to any other physical or mental disorder.

292.11 Hallucinogen Delusional Disorder

The essential feature is an Organic Delusional Syndrome (p. 114) that persists beyond the period of direct effect of the hallucinogen, i.e., 24 hours after cessation of hallucinogen use.

The Organic Delusional Syndrome emerges during or following a Hallucinogen Hallucinosiis (p. 153). All of the perceptual changes described in Hallucinogen Hallucinosiis may occur; but, in addition, the individual has a delusional conviction that the disturbed perceptions and thoughts correspond to reality.

Course. The course is variable, and may range from a brief, transitory experience to a long-lasting, psychotic episode that is difficult to distinguish from a nonorganic psychotic disorder, such as Schizophreniform Disorder or Acute Paranoid Disorder.

Complications. See Hallucinogen Hallucinosiis, p. 154.

Differential diagnosis. See Organic Delusional Syndrome, p. 114. A pre-existing **nonorganic psychotic disorder** should be considered if a Hallucinogen Delusional Disorder persists.

Diagnostic criteria for Hallucinogen Delusional Disorder

A. Recent hallucinogen use.

B. Development of an Organic Delusional Syndrome (p. 115) that persists beyond 24 hours after cessation of hallucinogen use.

C. Not due to any other physical or mental disorder, such as Schizophrenia.

292.84 Hallucinogen Affective Disorder

The essential feature is an Organic Affective Syndrome (p. 117) that persists beyond the period of direct effect of the hallucinogen, i.e., 24 hours after cessation of hallucinogen use.

Typically, the mood emerges shortly after hallucinogen use. Most common is the appearance of depression or anxiety; elation is rare. The depressive features often include feelings of self-reproach or excessive or inappropriate guilt, accompanied by fearfulness, tension, and physical restlessness. The individual may be unable to stop talking and have difficulty sleeping. Such individuals are frequently preoccupied with thoughts that they have destroyed their brains, that they have driven themselves crazy and will be unable to return to their normal state. These thoughts are without delusional conviction. When there is elation, grandiosity, decreased need for sleep, distractibility, increased activity, and loquacity are also present.

Course. The course is variable, and may range from a brief, transitory experience to a long-lasting episode that is difficult to distinguish from an Affective Disorder.

Complications. Manic and major depressive episodes.

Differential diagnosis. An individual with a preexisting **Affective Disorder** may take an hallucinogen to elevate his or her mood and then become more depressed. In such cases it may be difficult or impossible to determine if the increased disturbance in mood is due to Hallucinogen Affective Disorder or is merely an exacerbation of the Affective Disorder. See Organic Affective Syndrome, p. 117.

Hallucinogen Delusional Disorder may also be accompanied by marked affective changes. This diagnosis preempts a diagnosis of Hallucinogen Affective Disorder.

Diagnostic criteria for Hallucinogen Affective Disorder

A. Recent use of a hallucinogen.

B. Development of an Organic Affective Syndrome (p. 118) that persists beyond 24 hours after cessation of hallucinogen use.

C. Absence of delusions.

D. Not due to any other physical or mental disorder, such as pre-existing Affective Disorder.

CANNABIS ORGANIC MENTAL DISORDERS

This group includes all substances with psychoactive properties from the cannabis plant as well as chemically similar synthetic substances. In the United States

the most commonly used substances are marijuana, hashish, and, occasionally, purified delta-9-tetrahydrocannabinol (THC). These substances are smoked or taken orally in a number of forms, including mixed with food.

305.20 Cannabis Intoxication

The essential features are specific psychological and physical symptoms and maladaptive behavioral effects due to the recent use of cannabis.

The psychological symptoms include euphoria, subjective intensification of perceptions, the sensation of slowed time (5 minutes may seem like an hour), preoccupation with auditory or visual stimuli, and apathy. The euphoria may be expressed as a marked sense of well-being and relaxation. The individual may be indifferent to his or her surroundings.

Tachycardia is invariably present, although it is not as prominent when the individual is a chronic user of cannabis. Conjunctival injection is almost always present. Other physical symptoms include increased appetite, often for "junk food," and a dry mouth.

Maladaptive behavioral effects include paranoid ideation, panic attacks, and dysphoric affects. The individual may believe that he or she is dying or going crazy. (Some believe that adverse reactions are more likely to occur in individuals with rigid personalities, in individuals with a history of having had a psychotic disorder, or in circumstances considered to be threatening, such as the possibility of a police raid.) Other maladaptive behavioral effects include impaired judgment and interference with social or occupational functioning.

Associated features. Depersonalization or derealization may occur. Hallucinations are rare except when very high blood levels are reached. (In such circumstances, the substance acts as an hallucinogen. Because this rarely occurs, a separate diagnosis of Cannabis Hallucinosis is not included in this classification.)

Course. Intoxication occurs almost immediately after smoking marijuana, peaks within a half hour, and usually lasts about 3 hours. Orally ingested cannabis is more slowly absorbed and reaches a lower peak blood level, and the effects last longer.

Complications. Automobile accidents may occur because of impaired motor coordination.

Differential diagnosis. Other Substance-induced Intoxications should be considered. The individual with Cannabis Intoxication will sometimes have the characteristic sweet smell of burned cannabis on his or her clothing. **Alcohol Intoxication** frequently decreases appetite, increases aggressive behavior, produces nystagmus or ataxia, and is associated with the smell of alcohol, whereas these symptoms are rare in Cannabis Intoxication. **Hallucinogens** in low doses cause a clinical picture that resembles Cannabis Intoxication. If the administration was by smoking, this would indicate Cannabis Intoxication, since hallucinogens are not smoked. If administration was by ingestion, Cannabis Intoxication still must be considered.

Diagnostic criteria for Cannabis Intoxication

- A. Recent use of cannabis.
- B. Tachycardia.
- C. At least one of the following psychological symptoms within 2 hours of use:
 - (1) euphoria
 - (2) subjective intensification of perceptions
 - (3) sensation of slowed time
 - (4) apathy
- D. At least one of the following physical symptoms within 2 hours of substance use:
 - (1) conjunctival injection
 - (2) increased appetite
 - (3) dry mouth
- E. Maladaptive behavioral effects, e.g., excessive anxiety, suspiciousness or paranoid ideation, impaired judgment, interference with social or occupational functioning.
- F. Not due to any other physical or mental disorder.

292.11 Cannabis Delusional Disorder

The essential feature is an Organic Delusional Syndrome (p. 114), usually with persecutory delusions, immediately following cannabis use or during the course of Cannabis Intoxication.

The existence of this category is controversial since the disturbance does not persist beyond a few hours, the usual duration of Cannabis Intoxication.

Associated features. Marked anxiety, emotional lability, depersonalization, and subsequent amnesia for the episode can occur. Associated physical symptoms are the same as those seen in Cannabis Intoxication.

Differential diagnosis. See Organic Delusional Syndrome, p. 114.

Diagnostic criteria for Cannabis Delusional Disorder

- A. Recent use of cannabis.
- B. An Organic Delusional Syndrome (p. 115) within 2 hours of substance use.

- C. The disturbance does not persist beyond 6 hours following cessation of substance use.
- D. Not due to any other physical or mental disorder.

TOBACCO ORGANIC MENTAL DISORDER

292.00 Tobacco Withdrawal

The essential feature is a characteristic withdrawal syndrome due to recent cessation of or reduction in tobacco use that has been at least moderate in duration and amount. The syndrome includes craving for tobacco, irritability, anxiety, difficulty concentrating, restlessness, headache, drowsiness, and gastrointestinal disturbances. It is assumed that this syndrome is caused by nicotine withdrawal, since nicotine is the major pharmacologically active ingredient in tobacco.

Withdrawal does not occur with all smokers; but in many heavy cigarette smokers, changes in mood and performance that are probably related to withdrawal can be detected within 2 hours after the last cigarette. The sense of craving appears to reach a peak within the first 24 hours after the last cigarette, thereafter gradually declining over a few days to several weeks. In any given case it is difficult to distinguish between a withdrawal effect and the emergence of psychological traits that were suppressed, controlled, or altered by the effects of nicotine.

Associated features. Increased slow rhythms on the EEG, increased frequency of masseter muscle contractions, decreased heart rate and blood pressure, weight gain, and impairment in performance of tasks requiring vigilance are common associated features of Tobacco Withdrawal.

Course. The symptoms begin within 24 hours of cessation of or reduction in tobacco use and decrease in intensity over a period of a few days to several weeks.

Differential diagnosis. The diagnosis of Tobacco Withdrawal is usually self-evident from the individual's history, and the disappearance of symptoms if smoking is resumed is confirmatory.

Diagnostic criteria for Tobacco Withdrawal

- A. Use of tobacco for at least several weeks at a level equivalent to more than ten cigarettes per day, with each cigarette containing at least 0.5 mg of nicotine.
- B. Abrupt cessation of or reduction in tobacco use, followed within 24 hours by at least four of the following:

- (1) craving for tobacco
- (2) irritability
- (3) anxiety
- (4) difficulty concentrating
- (5) restlessness
- (6) headache
- (7) drowsiness
- (8) gastrointestinal disturbances

CAFFEINE ORGANIC MENTAL DISORDER

The consumption of caffeine, especially in the form of coffee, tea, cola, chocolate, and cocoa, is ubiquitous in the United States. Other common sources of caffeine are over-the-counter analgesics, "cold" preparations, and stimulants.

305.90 Caffeine Intoxication

The essential features are such characteristic effects of the recent use of caffeine-containing substances as restlessness, nervousness, excitement, insomnia, flushed face, diuresis, and gastrointestinal complaints. These symptoms appear in some individuals following ingestion of as little as 250 mg of caffeine per day, whereas others require much larger doses. At levels of more than 1 g/day there may be muscle twitchings, periods of inexhaustibility, psychomotor agitation, rambling flow of thought and speech, and cardiac arrhythmia. Mild sensory disturbances such as ringing in the ears and flashes of light have been reported at higher doses. With doses exceeding 10 g of caffeine, grand mal seizures and respiratory failure may result in death.

This disorder has been called Caffeinism.

A rough guide to calculating caffeine intake follows: coffee contains 100-150 mg of caffeine per cup; tea is about half as strong; a glass of cola is about a third as strong. Most caffeine-containing prescriptions and over-the-counter medications are one-third to one-half the strength of a cup of coffee. Two notable exceptions are migraine medications and over-the-counter stimulants that contain 100 mg per tablet.

Complications. Complications include developing or aggravating gastrointestinal and heart disease. Caffeine can produce epigastric distress and, occasionally, peptic ulcer and hematemesis. In addition to arrhythmia with extremely high dosages, the substance can cause marked hypotension and circulatory failure.

Differential diagnosis. Manic episodes, Panic Disorder or Generalized Anxiety Disorder can cause a clinical picture similar to that of Caffeine Intoxication. The temporal relationship of the symptoms to caffeine use establishes the diagnosis.

Diagnostic criteria for Caffeine Intoxication

A. Recent consumption of caffeine, usually in excess of 250 mg.

B. At least five of the following:

- (1) restlessness
- (2) nervousness
- (3) excitement
- (4) insomnia
- (5) flushed face
- (6) diuresis
- (7) gastrointestinal complaints
- (8) muscle twitching
- (9) rambling flow of thought and speech
- (10) cardiac arrhythmia
- (11) periods of inexhaustibility
- (12) psychomotor agitation

C. Not due to any other mental disorder, such as an Anxiety Disorder.

OTHER OR UNSPECIFIED SUBSTANCE-INDUCED ORGANIC MENTAL DISORDERS

This section is to be used when an individual develops an Organic Brain Syndrome apparently due to use of a substance if:

- (1) the substance cannot be classified in any of the ten previously listed categories (examples: Levo-dopa Delusional Disorder, Anticholinergic Delirium); or
- (2) the syndrome is caused by an unknown substance (example: an Intoxication after taking a bottle of unlabeled pills).

Following the listing of each of the diagnoses in this section, the reader is directed to the page listing the diagnostic criteria for the various Organic Brain Syndromes.

305.90 Other or Unspecified Substance Intoxication (p. 120)

292.00 Other or Unspecified Substance Withdrawal (p. 122)

292.81 Other or Unspecified Substance Delirium (p. 104)

292.82 Other or Unspecified Substance Dementia (p. 107)

292.83 Other or Unspecified Substance Amnestic Disorder (p. 112)

292.11 Other or Unspecified Substance Delusional Disorder (p. 114)

- 292.12 Other or Unspecified Substance Hallucinosis** (p. 115)
- 292.84 Other or Unspecified Substance Affective Disorder** (p. 117)
- 292.89 Other or Unspecified Substance Personality Disorder** (p. 118)
- 292.90 Other or Unspecified Substance Atypical or Mixed Organic Mental Disorder** (p. 123)

ORGANIC MENTAL DISORDERS—SECTION 2

Organic Mental Disorders in which the etiology or pathophysiological process is either noted on Axis III as an additional diagnosis from outside of the mental disorders section of ICD-9-CM or is unknown.

This section permits the identification of specific Organic Brain Syndromes on Axis I, associated with physical disorders noted on Axis III. Examples would include Delirium (Axis I) associated with pneumonia (Axis III) and Dementia (Axis I) associated with brain tumor (Axis III). Following the name of each of the Organic Brain Syndromes is the page listing the diagnostic criteria for the syndrome.

- 293.00 Delirium** (p. 107)
- 294.10 Dementia** (p. 111)
- 294.00 Amnestic Syndrome** (p. 113)
- 293.81 Organic Delusional Syndrome** (p. 115)
- 293.82 Organic Hallucinosiis** (p. 116)
- 293.83 Organic Affective Syndrome** (p. 118)
- 310.10 Organic Personality Syndrome** (p. 119)
- 294.80 Atypical or Mixed Organic Brain Syndrome** (p. 123)

Substance Use Disorders

In our society, use of certain substances to modify mood or behavior under certain circumstances is generally regarded as normal and appropriate. Such use includes recreational drinking of alcohol, in which a majority of adult Americans participate, and the use of caffeine as a stimulant in the form of coffee. On the other hand, there are wide subcultural variations. In some groups even the recreational use of alcohol is frowned upon, while in other groups the use of various illegal substances for recreational purposes is widely accepted. In addition, certain substances are used medically for the alleviation of pain, relief of tension, or to suppress appetite.

This diagnostic class deals with behavioral changes associated with more or less regular use of substances that affect the central nervous system. These behavioral changes in almost all subcultures would be viewed as extremely undesirable. Examples of such behavioral changes include impairment in social or occupational functioning as a consequence of substance use, inability to control use of or to stop taking the substance, and the development of serious withdrawal symptoms after cessation of or reduction in substance use. These conditions are here conceptualized as mental disorders and are therefore to be distinguished from nonpathological substance use for recreational or medical purposes.

The disorders classified in this section are to be distinguished from the corresponding portions of the Organic Mental Disorders section. Whereas the Substance Use Disorders refer to the maladaptive behavior associated with more or less regular use of the substances, the Substance-induced Organic Mental Disorders describe the direct acute or chronic effects of these substances on the central nervous system. Almost invariably, individuals who have a Substance Use Disorder will also at various times have a Substance-induced Organic Mental Disorder, such as an Intoxication or Withdrawal.

For most classes of substances, pathological use is divided into Substance Abuse and Substance Dependence, defined below:

SUBSTANCE ABUSE

Pattern of pathological use

Impairment in social or occupational functioning due to substance use

Minimal duration of disturbance of at least one month

SUBSTANCE DEPENDENCE

Tolerance or withdrawal

(For Alcohol Dependence and Cannabis Dependence a pattern of pathological use or impairment in social or occupational functioning is also required. For the exception of Tobacco Dependence, see p. 176.)

SUBSTANCE ABUSE

Three criteria distinguish nonpathological substance use from Substance Abuse.

A pattern of pathological use. Depending upon the substance, this may be manifested by: intoxication throughout the day, inability to cut down or stop use, repeated efforts to control use through periods of temporary abstinence or restriction of use to certain times of the day, continuation of substance use despite a serious physical disorder that the individual knows is exacerbated by use of the substance, need for daily use of the substance for adequate functioning, and episodes of a complication of the substance intoxication (e.g., alcoholic blackouts, opioid overdose).

Impairment in social or occupational functioning caused by the pattern of pathological use. Social relations can be disturbed by the individual's failure to meet important obligations to friends and family, by display of erratic and impulsive behavior, and by inappropriate expression of aggressive feelings. The individual may have legal difficulties because of complications of the intoxicated state (e.g., car accidents) or because of criminal behavior to obtain money to purchase the substance. (However, legal difficulties due to possession, purchase, or sale of illegal substances are highly dependent on local customs and laws, and change over time. For this reason, such legal difficulty on a single occasion should not be considered in the evaluation of impairment in social functioning for diagnostic purposes.)

Occupational functioning can deteriorate if the individual misses work or school, or is unable to function effectively because of being intoxicated. When impairment is severe, the individual's life can become totally dominated by use of the substance, with marked deterioration in physical and psychological functioning. Incapacitation is more frequently associated with chronic Opioid and Alcohol Dependence than with dependence on other substances.

Frequently individuals who develop Substance Use Disorders also have preexisting Personality Disorders and Affective Disorders with concomitant impairment in social and occupational functioning. It is therefore necessary to determine that the social or occupational impairment associated with the diagnosis of Substance Abuse or Dependence is actually due to the use of the substance. The best clue is a change in functioning that accompanies the onset of a pathological pattern of substance use, or the development of physiological dependence.

Duration. Abuse as used in this manual requires that the disturbance last at least *one month*. Signs of the disturbance need not be present continuously throughout the month, but should be sufficiently frequent for a *pattern* of pathological use causing interference with social or occupational functioning to be apparent. For example, several episodes of binge drinking causing family arguments during a one-month period would be sufficient even though between binges the individual's functioning was apparently not impaired.

Isolated instances of pathological use of a substance can be adequately diagnosed by noting the specific Organic Brain Syndromes that were associated with this use. For example, a history of one or more instances of maladaptive use of alcohol over a three-week period may be noted as prior episodes of Alcohol Intoxication.

SUBSTANCE DEPENDENCE

Substance Dependence generally is a more severe form of Substance Use Disorder than Substance Abuse and requires physiological dependence, evidenced by either tolerance or withdrawal. Almost invariably there is also a pattern of pathological use that causes impairment in social or occupational functioning, although in rare cases the manifestations of the disorder are limited to physiological dependence. An example would be an individual's inadvertently becoming physiologically dependent on an analgesic opioid given to him by a physician for the relief of physical pain.

The diagnosis of all of the Substance Dependence categories requires only evidence of tolerance or withdrawal, except for Alcohol and Cannabis Dependence, which in addition require evidence of social or occupational impairment from use of the substance or a pattern of pathological substance use.

Tolerance. Tolerance means that markedly increased amounts of the substance are required to achieve the desired effect or there is a markedly diminished effect with regular use of the same dose. When the substance used is illegal and mixed with various diluents or with other substances, tolerance may be difficult to determine. In the case of alcohol, it should be noted that there are wide individual variations in the capacity to drink large quantities of alcohol without intoxication. Since some persons have the capacity to drink large amounts despite limited drinking experience, the distinguished feature of tolerance is that the individual reports that the amount of alcohol he or she can drink before showing signs of intoxication has increased markedly over time.

Withdrawal. In withdrawal, a substance-specific syndrome follows cessation of or reduction in intake of a substance that was previously regularly used by the individual to induce a physiological state of intoxication. See Withdrawal as an Organic Brain Syndrome, p. 122.

Many heavy coffee drinkers are physiologically dependent on caffeine and exhibit both tolerance and withdrawal. However, since such use generally does not cause distress or social or occupational impairment, and since few if any of these individuals have difficulty switching to decaffeinated coffee or coffee substitutes, the condition does not appear to be of clinical significance. Therefore, caffeine dependence is not included in this classification of mental disorders. In contrast, Caffeine Intoxication is often clinically significant, and therefore is included (p. 160).

CLASSES OF SUBSTANCES

Five classes of substances are associated with both abuse and dependence: alcohol, barbiturates or similarly acting sedatives or hypnotics, opioids, amphet-

mines or similarly acting sympathomimetics, and cannabis. Some of these substances are used medically, such as the amphetamines, barbiturates, and opioids. Three classes of substances are associated only with abuse because physiological dependence has not been demonstrated: cocaine, phencyclidine (PCP) or similarly acting arylcyclohexylamines, and hallucinogens. Finally, one substance, tobacco, is associated only with dependence, since heavy use of tobacco itself is not associated with impairment in social or occupational functioning (though the reaction of others to the tobacco use may cause difficulties).

USE OF MULTIPLE SUBSTANCES

Substance Abuse and Dependence frequently involve several substances. Individuals with Barbiturate or Similarly Acting Sedative or Hypnotic Abuse or Dependence often may also have problems with alcohol or, more rarely, use amphetamine to counter sedative effects. Individuals with Opioid or Cannabis Abuse or Dependence usually have several other Substance Use Disorders, particularly of barbiturates or similarly acting sedatives and hypnotics, amphetamines or similarly acting sympathomimetics, and cocaine.

When an individual's condition meets the criteria for more than one Substance Use Disorder, multiple diagnoses should generally be made. (The exception to this is when the abuse or dependence involves so many substances that the clinician prefers to indicate a combination of substances rather than list each specific substance. See p. 179.)

RECORDING SPECIFIC DIAGNOSES

The clinician should record the name of the specific substance rather than of the entire class of substances, using the code number for the appropriate class. Examples: The clinician should write 305.73 Amphetamine Abuse, In Remission (rather than Amphetamine or Similarly Acting Sympathomimetic Abuse); 304.11 Valium Dependence, Continuous (rather than Barbiturate or Similarly Acting Sedative or Hypnotic Dependence); 305.91 Compazine Abuse, Continuous (rather than Other, Mixed, or Unspecified Substance Abuse).

SUBCLASSIFICATION OF COURSE

No entirely adequate method for subtyping the course of these disorders is available. However, the following guidelines should be used to indicate the course of the illness in the fifth digit:

Code	Course	Definition
1	Continuous	More or less regular maladaptive use for over six months.
2	Episodic	A fairly circumscribed period of maladaptive use, with one or more similar periods in the past.
3	In remission	Previous maladaptive use, but not using substance at present. The differentiation of this from no longer ill and from the other course categories requires consideration of the period of time since the last period of disturbance, the total duration of the

Code	Course	Definition
0	Unspecified	disturbance, and the need for continued evaluation or prophylactic treatment. Course unknown or first signs of illness with course uncertain.

OTHER FEATURES OF SUBSTANCE USE DISORDERS

Associated features. Personality disturbance and disturbance of mood are often present, and may be intensified by the Substance Use Disorder. For example, antisocial personality traits may be accentuated by the need to obtain money to purchase illegal substances. Anxiety or depression associated with Borderline Personality Disorder may be intensified as the individual uses a substance in an unsuccessful attempt to treat his or her mood disturbance.

Abuse of certain substances, particularly cocaine, hallucinogens, or cannabis, may be associated with identification with countercultural life-styles or, more rarely, identification with non-traditional religious or mystical ideas.

In chronic Abuse and Dependence, mood lability and suspiciousness, both of which can contribute to violent behavior, are common.

Age at onset. Alcohol Abuse and Dependence usually appear in the 20s, 30s, and 40s. Opioid, Cocaine, Amphetamine or Similarly Acting Sympathomimetic, Hallucinogen, Cannabis, Cocaine, Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine, and Tobacco Use Disorders more commonly begin in the late teens and 20s. Two patterns of onset for Barbiturate or Similarly Acting Sedative or Hypnotic Abuse and Dependence have been identified (p. 171). When a Substance Use Disorder begins early in life, it is often associated with failure to complete school and a lifelong pattern of low occupational achievement.

Complications. The abuse or dependence associated with each class of substance may cause an Organic Brain Syndrome. For example, prolonged Alcohol Dependence may cause Alcohol Withdrawal Delirium, Alcohol Amnestic Disorder, or Alcohol Hallucinoses. Similarly, Hallucinogen Delusional Disorder may be a complication of chronic hallucinogen use. Complications of the specific intoxication states, such as traffic accidents and physical injury due to Alcohol Intoxication, have been noted in the Organic Mental Disorders section.

Frequently there is a deterioration in the general level of physical health. Malnutrition and a variety of other physical disorders may result from failure to maintain physical health by proper diet and adequate personal hygiene.

Use of contaminated needles for the intravenous administration of opioids, cocaine, and amphetamines can cause hepatitis, tetanus, vasculitis, septicemia, subacute bacterial endocarditis, embolic phenomena, or malaria. Materials used to "cut" the substances can cause toxic or allergic reactions. Use of cocaine by means of the intranasal route ("snorting") sometimes results in erosion of the nasal septum.

Physical complications of chronic Alcohol Dependence include hepatitis, cirrhosis, peripheral neuropathy, and gastritis. In addition, chronic Alcohol Dependence increases the risk and seriousness of heart disease, pneumonia, tuberculosis, and neurological disorders. The long-term potential for respiratory disorder in chronic cannabis use is controversial. The long-term physical complications of chronic and heavy tobacco use are discussed on p. 177.

Depressive symptoms are a frequent complication of Substance Use Disorders, and partly account for the high rate of suicide by individuals with Substance Dependence. Suicide associated with alcohol and other substances can occur in both intoxicated and sober states.

Predisposing factors. Personality Disorders, particularly Antisocial Personality Disorder, predispose to the development of Substance Use Disorders.

Prevalence. Most of the Substance Use Disorders are common, especially those associated with alcohol and tobacco. Others, such as Opioid Abuse, are rare. For example, about 16% of the American public report *some* problem associated with alcohol within the past three years, and about 4% report more than trivial problems. In some economically deprived urban communities, Opioid Abuse and Dependence are widespread.

Sex ratio. Substance Use Disorders are diagnosed more commonly in men than in women.

Differential diagnosis. **Nonpathological substance use for recreational or medical purposes** is not associated with impairment in social or occupational functioning or a pathological pattern of use.

Repeated episodes of substance-induced intoxication are almost invariably present in Substance Abuse or Dependence, although for some substances it is possible to develop dependence without ever exhibiting frank intoxication (e.g., alcohol). Furthermore, substance-induced intoxication as an isolated episode not involving either abuse or dependence is common.

There are now methods to detect the presence of alcohol, barbiturates and similarly acting sedatives and hypnotics, opioids, cocaine, and amphetamines in serum or urine. In some cases the tests indicate that an individual who thinks he or she has been using one substance, such as cocaine, has in fact been taking something else, such as amphetamine crystals.

A test dose may be used to establish tolerance to barbiturates by administering 200 mg of a short-acting barbiturate (usually pentobarbital) hourly until early signs of intoxication appear. The total amount of barbiturate required to produce these signs of intoxication is multiplied by a factor of three, giving an approximation of the individual's daily tolerance level to barbiturates.

When Opioid Dependence is suspected, signs and symptoms of Opioid Withdrawal may be precipitated by the subcutaneous administration of 0.4 mg of naloxone, an opioid antagonist. (This should not be administered to individuals with a history of cardiac disease or coronary insufficiency.)

305.0x Alcohol Abuse**303.9x Alcohol Dependence**

For a description of Alcohol Intoxication, Alcohol Withdrawal, and the other Alcohol Organic Mental Disorders, see Alcohol-induced Organic Mental Disorders, p. 129.

The essential feature of Alcohol Abuse is a pattern of pathological use for at least a month that causes impairment in social or occupational functioning.

The essential features of Alcohol Dependence are either a pattern of pathological alcohol use or impairment in social or occupational functioning due to alcohol, and either tolerance or withdrawal. Alcohol Dependence has also been called Alcoholism.

Course. When abuse or dependence develops, it is usually within the first five years after regular drinking is established. Heavy drinking in adolescence (before age 16) is particularly likely to be associated with later problems.

Although Alcohol Dependence and Abuse can continue into old age, they may remit with aging, sometimes in response to the development of physical complications. Occasional drinking with rare or no episodes of intoxication *does* occur in some persons with a clear prior history of Alcohol Dependence. Therefore, drinking that is currently moderate should not be considered evidence for the absence of Alcohol Dependence in the past.

There are three main patterns of chronic pathological alcohol use. The first is regular daily intake of large amounts; the second is regular heavy drinking limited to weekends. These two patterns are included in the fifth-digit subtype "Continuous." The third pattern is long periods of sobriety interspersed with binges of daily heavy drinking lasting for weeks or months. This pattern corresponds to the fifth-digit subtype "Episodic."

Familial pattern. Alcohol Abuse and Dependence are more common among family members than in the general population. Evidence of a genetic factor is the increased prevalence of Alcohol Dependence in the early-adopted offspring of parents with the disorder.

Diagnostic criteria for Alcohol Abuse

A. *Pattern of pathological alcohol use:* need for daily use of alcohol for adequate functioning; inability to cut down or stop drinking; repeated efforts to control or reduce excess drinking by "going on the wagon" (periods of temporary abstinence) or restricting drinking to certain times of the day; binges (remaining intoxicated throughout the day for at least two days); occasional consumption of a fifth of spirits (or its equivalent in wine or beer); amnesic periods for events occurring while intoxicated (blackouts); continuation of drinking despite a serious physical disorder that the individual knows is exacerbated by alcohol use; drinking of non-beverage alcohol.

B. *Impairment in social or occupational functioning due to alcohol use:* e.g., violence while intoxicated, absence from work, loss of job, legal difficulties (e.g., arrest for intoxicated behavior, traffic accidents while intoxicated), arguments or difficulties with family or friends because of excessive alcohol use.

C. Duration of disturbance of at least one month.

Diagnostic criteria for Alcohol Dependence

A. Either a pattern of pathological alcohol use or impairment in social or occupational functioning due to alcohol use:

Pattern of pathological alcohol use: need for daily use of alcohol for adequate functioning; inability to cut down or stop drinking; repeated efforts to control or reduce excess drinking by “going on the wagon” (periods of temporary abstinence) or restricting drinking to certain times of the day; binges (remaining intoxicated throughout the day for at least two days); occasional consumption of a fifth of spirits (or its equivalent in wine or beer); amnesic periods for events occurring while intoxicated (blackouts); continuation of drinking despite a serious physical disorder that the individual knows is exacerbated by alcohol use; drinking of non-beverage alcohol.

Impairment in social or occupational functioning due to alcohol use: e.g., violence while intoxicated, absence from work, loss of job, legal difficulties (e.g., arrest for intoxicated behavior, traffic accidents while intoxicated), arguments or difficulties with family or friends because of excessive alcohol use.

B. Either tolerance or withdrawal:

Tolerance: need for markedly increased amounts of alcohol to achieve the desired effect, or markedly diminished effect with regular use of the same amount.

Withdrawal: development of Alcohol Withdrawal (e.g., morning “shakes” and malaise relieved by drinking) after cessation of or reduction in drinking (p. 133).

305.4x Barbiturate or Similarly Acting Sedative or Hypnotic Abuse

304.1x Barbiturate or Similarly Acting Sedative or Hypnotic Dependence

See Barbiturate or Similarly Acting Sedative or Hypnotic Organic Mental Disorders (p. 139) for a discussion of the substances included in this class, usual routes of administration, and a description of Barbiturate or Similarly Acting Sedative or Hypnotic Intoxication, Withdrawal, and Amnesic Disorder.

The essential feature of Barbiturate or Similarly Acting Sedative or Hypnotic Abuse is a pattern of pathological use for at least one month that

causes impairment in social or occupational functioning. The essential feature of Barbiturate or Similarly Acting Sedative or Hypnotic Dependence is either tolerance or withdrawal.

There are two patterns of development of dependence and abuse. In one, the individual has originally obtained the substance by prescription from a physician for insomnia, but has gradually increased the dose and frequency of use on his or her own. At first this increase is intended for sleep, but the individual then discovers that adjusted doses of the substance during the day seem to help in coping with daily living problems through relief of tension and anxiety. Individuals with this pattern are more apt to be from a middle-class background, between ages 30-60, and female. The other pattern involves individuals who are apt to be males in their teens or early 20s who, with a group of peers, use substances obtained from illegal sources. The initial objective is to attain a "high" or euphoria, or to counteract the stimulant effects of amphetamines.

Course. The most common course is heavy daily use that results in dependence. A significant number of individuals with dependence eventually stop using the substance, and demonstrate a permanent recovery, even from the physical complications of the disorder.

Diagnostic criteria for Barbiturate or Similarly Acting Sedative or Hypnotic Abuse

A. *Pattern of pathological use:* inability to cut down or stop use; intoxication throughout the day; frequent use of the equivalent of 600 mg or more of secobarbital or 60 mg or more of diazepam; amnesic periods for events that occurred while intoxicated.

B. *Impairment in social or occupational functioning due to substance use:* e.g., fights, loss of friends, absence from work, loss of job, or legal difficulties (other than a single arrest due to possession, purchase, or sale of the substance).

C. Duration of disturbance of at least one month.

Diagnostic criteria for Barbiturate or Similarly Acting Sedative or Hypnotic Dependence

Either tolerance or withdrawal:

Tolerance: need for markedly increased amounts of the substance to achieve the desired effect, or markedly diminished effect with regular use of the same amount.

Withdrawal: development of Barbiturate or Similarly Acting Sedative or Hypnotic Withdrawal (p. 140) after cessation of or reduction in substance use.

305.5x Opioid Abuse**304.0x Opioid Dependence**

See Opioid Organic Mental Disorders (p. 142) for a discussion of the substances included in this class, usual routes of administration, and a description of Opioid Intoxication and Withdrawal.

The essential feature of Opioid Abuse is a pattern of pathological use for at least one month that causes impairment in social or occupational functioning.

The essential feature of Opioid Dependence is either tolerance or withdrawal.

Course. Opioid Abuse and Dependence are generally preceded by a period of "polydrug use," which may involve tobacco, alcohol, marijuana, sedative-hypnotics, prescription and nonprescription cough syrups, hallucinogens, or amphetamines. The use of these other substances usually continues after the use of opioids is established. Once a pattern of Opioid Dependence is established, substance procurement and use usually dominates the individual's life.

Approximately half of the individuals who engage in Opioid Abuse go on to develop Opioid Dependence. Once Opioid Dependence is established, the course is a function of the context of the addiction. For example, the vast majority of persons who became dependent on heroin in Vietnam did not return to their addiction when back in the United States. In contrast, it is believed that most individuals who become dependent on opioids in the United States become involved in a chronic behavioral disorder, marked by remissions while in treatment or prison or when the substance is scarce and relapses on returning to a familiar environment where these substances are available and friends or colleagues use these substances. In the United States, in this century, persons with Opioid Dependence have a high annual death rate (approximately 10 per 1,000) because of the physical complications of the disorder and a lifestyle often associated with violence. Among those who survive, increased abstinence is found with the passage of years, with final cessation of dependence an average of about nine years after its onset.

Diagnostic criteria for Opioid Abuse

A. *Pattern of pathological use:* inability to reduce or stop use; intoxication throughout the day; use of opioids nearly every day for at least a month; episodes of opioid overdose (intoxication so severe that respiration and consciousness are impaired).

B. *Impairment in social or occupational functioning due to opioid use:* e.g., fights, loss of friends, absence from work, loss of job, or legal difficulties (other than due to a single arrest for possession, purchase, or sale of the substance).

C. Duration of disturbance of at least one month.

Diagnostic criteria for Opioid Dependence

Either tolerance or withdrawal:

Tolerance: need for markedly increased amounts of opioid to achieve the desired effect, or markedly diminished effect with regular use of the same amount.

Withdrawal: development of Opioid Withdrawal (p. 144) after cessation of or reduction in substance use.

305.6x Cocaine Abuse

See Cocaine Organic Mental Disorders (p. 145) for a discussion of the usual routes of administration and a description of Cocaine Intoxication. Since only transitory withdrawal symptoms occur after cessation of or reduction in prolonged use, a separate category of dependence on cocaine is not included.

The essential feature of Cocaine Abuse is a pathological pattern of use for at least one month that causes impairment in social or occupational functioning.

Course. The development of Cocaine Abuse may take two to eight months of cocaine use. Paranoid ideation, suspiciousness, and ritualistic behavior generally occur late in the course of abuse. Although habitual use has lasted 10 to 15 years in some persons, Cocaine Abuse does not have as prolonged a course as is usually the case with Barbiturate and Opioid Abuse.

Diagnostic criteria for Cocaine Abuse

A. *Pattern of pathological use*: inability to reduce or stop use; intoxication throughout the day; episodes of cocaine overdose (intoxication so severe that hallucinations and delusions occur in a clear sensorium).

B. *Impairment in social or occupational functioning due to cocaine use*: e.g., fights, loss of friends, absence from work, loss of job, or legal difficulties (other than due to a single arrest for possession, purchase, or sale of the substance).

C. Duration of disturbance of at least one month.

305.7x Amphetamine or Similarly Acting Sympathomimetic Abuse

304.4x Amphetamine or Similarly Acting Sympathomimetic Dependence

See Amphetamine or Similarly Acting Sympathomimetic Organic Mental Disorders (p. 147) for a discussion of the substances included in this class, usual routes of administration, and a description of Amphetamine or Similarly Acting Sympathomimetic Intoxication, Delirium, Delusional Disorder, and Withdrawal.

The essential feature of Amphetamine or Similarly Acting Sympathomimetic Abuse is a pattern of pathological use for at least a month that causes impairment in social or occupational functioning. The usual pattern is "runs" of daily use for 10 to 14 days at a time. The essential feature of Amphetamine or Similarly Acting Sympathomimetic Dependence is either tolerance or withdrawal.

Course. See Cocaine Abuse, p. 173.

Diagnostic criteria for Amphetamine or Similarly Acting Sympathomimetic Abuse

A. *Pattern of pathological use:* inability to reduce or stop use; intoxication throughout the day; use of substance nearly every day for at least one month; episodes of either Amphetamine or Similarly Acting Sympathomimetic Delusional Disorder or Amphetamine or Similarly Acting Sympathomimetic Delirium.

B. *Impairment in social or occupational functioning due to amphetamine or similarly acting sympathomimetic use:* e.g., fights, loss of friends, absence from work, loss of job, or legal difficulties (other than due to a single arrest for possession, purchase, or sale of the substance).

C. Duration of disturbance of at least one month.

Diagnostic criteria for Amphetamine or Similarly Acting Sympathomimetic Dependence

Either tolerance or withdrawal:

Tolerance: need for markedly increased amounts of substance to achieve the desired effect, or markedly diminished effect with regular use of the same amount.

Withdrawal: development of Amphetamine or Similarly Acting Sympathomimetic Withdrawal (p. 150) after cessation of or reduction in substance use.

305.9x Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Abuse

See Phencyclidine or Similarly Acting Arylcyclohexylamine Organic Mental Disorders (p. 150) for a discussion of the substances included in this class, usual routes of administration, and a description of Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Intoxication, Delirium, and Mixed Organic Mental Disorder.

The essential feature of Phencyclidine or Similarly Acting Arylcyclohexylamine Abuse is a pattern of pathological use for at least a month that causes impairment in social or occupational functioning.

Because no clear withdrawal syndrome or tolerance to this substance has been produced experimentally or observed clinically, a category for dependence is not included.

Course. See Cocaine Abuse, p. 173.

Diagnostic criteria for Phencyclidine (PCP) or Similarly Acting Arylcyclohexylamine Abuse

A. *Pattern of pathological use:* intoxication throughout the day; episodes

of Phencyclidine or Similarly Acting Arylcyclohexylamine Delirium or Mixed Organic Mental Disorder.

B. Impairment in social or occupational functioning due to substance use: e.g., fights, loss of friends, absence from work, loss of job, or legal difficulties (other than due to a single arrest for possession, purchase, or sale of the substance).

C. Duration of disturbance of at least one month.

305.3x Hallucinogen Abuse

See Hallucinogen Organic Mental Disorders (p. 153) for a discussion of the substances included in this class, usual route of administration, and a description of Hallucinogen Intoxication, Delusional Disorder, and Affective Disorder.

The essential feature of Hallucinogen Abuse is a pattern of pathological use for at least one month that causes impairment in social or occupational functioning.

Because no clear withdrawal syndrome has been produced experimentally or observed clinically, a category for dependence is not included.

Course. The course is unpredictable and is probably related to the nature of the underlying pathology that played a role in the onset of use. Most individuals rapidly resume their former life-style after only a brief period of abuse.

Diagnostic criteria for Hallucinogen Abuse

A. Pattern of pathological use: inability to reduce or stop use; intoxication throughout the day (possible only with some hallucinogens); episodes of Hallucinogen Delusional Disorder or Hallucinogen Affective Disorder.

B. Impairment in social or occupational functioning due to hallucinogen use: e.g., fights, loss of friends, absence from work, loss of job, or legal difficulties (other than due to a single arrest for possession, purchase, or sale of the illegal substance).

C. Duration of disturbance of at least one month.

305.2x Cannabis Abuse

304.3x Cannabis Dependence

See Cannabis Organic Mental Disorders (p. 156) for a discussion of the substances included in this class, usual routes of administration, and a description of Cannabis Intoxication and Delusional Disorder.

The essential feature of Cannabis Abuse is a pattern of pathological use

for at least a month that causes impairment in social or occupational functioning. The essential features of Cannabis Dependence are impairment in social or occupational functioning due to cannabis use, and tolerance (withdrawal has not been conclusively demonstrated). The existence and significance of tolerance (Cannabis Dependence) with regular heavy use of cannabis are controversial.

Course. Many users stop or decrease their use of cannabis spontaneously or when impairment in functioning develops.

Diagnostic criteria for Cannabis Abuse

A. *Pattern of pathological use:* intoxication throughout the day; use of cannabis nearly every day for at least a month; episodes of Cannabis Delusional Disorder.

B. *Impairment in social or occupational functioning due to cannabis use:* e.g., marked loss of interest in activities previously engaged in, loss of friends, absence from work, loss of job, or legal difficulties (other than due to a single arrest for possession, purchase, or sale of the substance).

C. Duration of disturbance of at least one month.

Diagnostic criteria for Cannabis Dependence

A. Either a pattern of pathological use or impairment in social or occupational functioning due to cannabis use.

Pattern of pathological use: intoxication throughout the day; use of cannabis nearly every day for at least a month; episodes of Cannabis Delusional Disorder.

Impairment in social or occupational functioning due to cannabis use: e.g., marked loss of interest in activities previously engaged in, loss of friends, absence from work, loss of job, or legal difficulties (other than a single arrest due to possession, purchase, or sale of an illegal substance).

B. *Tolerance:* need for markedly increased amounts of cannabis to achieve the desired effect or markedly diminished effect with regular use of the same amount.

305.1x Tobacco Dependence

The essential features are continuous use of tobacco for at least one month with either (1) unsuccessful attempts to stop or significantly reduce the amount of tobacco use on a permanent basis, (2) the development of Tobacco Withdrawal, or (3) the presence of a serious physical disorder (e.g., respiratory or cardiovascular disease) that the individual knows is exacerbated by tobacco use. In practice this diagnosis will be given only when either the individual is seeking professional help to stop smoking, or, in the judgment of the diagnosti-

cian, the use of tobacco is seriously affecting the individual's physical health. It should also be noted that a heavy smoker who has never tried to stop smoking, who has never developed Tobacco Withdrawal, and who has no tobacco-related serious physical disorder, according to the criteria in this manual, does not have the disorder of Tobacco Dependence, even though physiologically the individual is almost certainly dependent on tobacco.

At present, the most common form of Tobacco Dependence is associated with the inhalation of cigarette smoke. Pipe and cigar smoking, the use of snuff, and the chewing of tobacco are less likely to lead to Tobacco Dependence for several reasons. First of all, the probability of developing serious health complications is lower than with cigarette smoking, probably because relatively little smoke is inhaled; consequently, there is less distress arising from health concerns about the need to take the substance repeatedly. Second, the more rapid onset of nicotine effects with cigarette smoking leads to a more intensive habit pattern that is more difficult to give up owing to the frequency of reinforcement or the greater physical dependence on nicotine.

In recent years the evidence that tobacco use predisposes to a variety of serious physical disorders has led many individuals who are heavy smokers to attempt to give up the habit. However, many are unable to stop at all or, if they do, often resume tobacco use within a matter of months.

The difficulty in giving up tobacco use on a long-term basis, particularly with cigarettes, may be due to the unpleasant nature of the withdrawal syndrome, the highly overlearned nature of the habit that stems from the repeated effects of nicotine, which rapidly follow the inhalation of cigarette smoke (75,000 puffs per year for a pack-a-day smoker), and the likelihood that a desire to use tobacco is elicited by environmental cues, such as the ubiquitous presence of other smokers and the widespread availability of cigarettes.

When efforts to give up smoking are made, Tobacco Withdrawal may develop. (For a discussion of Tobacco Withdrawal, see p. 159.)

The most common tobacco-related serious physical disorders are bronchitis, emphysema, coronary artery disease, peripheral vascular disease, and a variety of cancers. The diagnostician must assess the role of tobacco use as an etiological or exacerbating factor for the particular individual after considering both individual circumstances and the latest available scientific information. If the individual with a serious case of one of the tobacco-related physical disorders continues to use tobacco, despite awareness of its harmful effects, a reasonable inference can be made that the individual is dependent on tobacco.

Associated features. Individuals with this disorder are frequently distressed at their inability to stop tobacco use, particularly when they have serious physical symptoms that are aggravated by tobacco use. Some individuals who are dependent on tobacco may have difficulty remaining in social or occupational situations that prohibit smoking.

Age at onset. Tobacco Dependence usually begins in late adolescence or early adult life.

Course. The course of Tobacco Dependence is variable. Some individuals repeatedly attempt to give up tobacco use without success. Others have a brief course, in that when they experience concern about tobacco use they make a prompt effort to stop and are successful in total cessation, although in many cases they may experience a period of Tobacco Withdrawal lasting from days to weeks (see p. 159). Studies of treatment outcome suggest that the relapse rate is greater than 50% in the first six months, and approximately 70% within the first twelve months. After a year's abstinence subsequent relapse is unlikely.

Impairment. Since tobacco use rarely causes any identifiable state of intoxication as does alcohol, there is no impairment in social or occupational functioning as an immediate and direct consequence of tobacco use.

Prevalence and sex ratio. A large proportion of the adult population of the United States uses tobacco, the prevalence among men being greater than that among women. Among teenage smokers, boys are affected approximately as often as girls. The prevalence of Tobacco Dependence as defined here is not known. Some individuals give up smoking as they grow older without ever meeting the criteria for the disorder. However, since surveys have shown that approximately 50% of smokers express a desire to be able to stop and are unable to do so, and since serious physical disorders that are aggravated by smoking are common, Tobacco Dependence is obviously widespread.

Familial pattern. Cigarette smoking among family members of individuals with Tobacco Dependence is more common than in the general population. However, the evidence for a genetic factor is extremely weak.

Differential diagnosis. The major differential diagnostic problems will be to determine whether or not a particular physical disorder, in an individual who is a heavy smoker, is exacerbated by tobacco use, and how long a period of abstinence from tobacco use justifies the judgment that the disorder is no longer present or is in a state of remission.

Diagnostic criteria for Tobacco Dependence

A. Continuous use of tobacco for at least one month.

B. At least one of the following:

- (1) serious attempts to stop or significantly reduce the amount of tobacco use on a permanent basis have been unsuccessful
- (2) attempts to stop smoking have led to the development of Tobacco Withdrawal (see p. 159)
- (3) the individual continues to use tobacco despite a serious physical disorder (e.g., respiratory or cardiovascular disease) that he or she knows is exacerbated by tobacco use

305.9x Other, Mixed, or Unspecified Substance Abuse

Other Substance Abuse should be recorded if a substance abused cannot be classified in any of the categories noted above, e.g., glue (inhalants), amyl nitrite.

Mixed Substance Abuse should be noted when the substances abused are from more than one nonalcoholic substance category, e.g., amphetamines and barbiturates. This category should be used only when the specific substances cannot be identified or when the abuse involves so many substances that the clinician prefers to indicate a combination of substances rather than list each specific substance.

Unspecified Substance Abuse should be recorded when a substance abused is unknown.

304.6x Other Specified Substance Dependence

This category should be used when the individual is dependent on a substance that cannot be classified in any of the previous categories, e.g., codeine or corticosteroids.

304.9x Unspecified Substance Dependence

This diagnosis can be used as an initial diagnosis in cases in which the specific substance is not yet known.

304.7x Dependence on a Combination of Opioid and Other Nonalcoholic Substances

This category should be used when the individual is dependent on both an opioid and a nonopioid nonalcoholic substance. An example might be dependence on both heroin and barbiturates. This category should be used only when the specific substances cannot be identified or when the dependence involves so many substances that the clinician prefers to indicate a combination of substances rather than list each specific substance.

304.8x Dependence on a Combination of Substances, Excluding Opioids and Alcohol

This category should be used when the individual is dependent on two or more nonopioid nonalcoholic substances. An example might be dependence on both amphetamines and barbiturates. This category should be used only when the specific substances cannot be identified or when the dependence involves so many substances that the clinician prefers to indicate a combination of substances rather than list each specific substance.

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Schizophrenic Disorders

The essential features of this group of disorders* are: the presence of certain psychotic features during the active phase of the illness, characteristic symptoms involving multiple psychological processes, deterioration from a previous level of functioning, onset before age 45, and a duration of at least six months. The disturbance is not due to an Affective Disorder or Organic Mental Disorder. At some phase of the illness Schizophrenia always involves delusions, hallucinations, or certain disturbances in the form of thought.

The limits of the concept of Schizophrenia are unclear. Some approaches to defining the concept have emphasized the tendency toward a deteriorating course (Kraepelin), underlying disturbances in certain psychological processes (Bleuler), or pathognomonic symptoms (Schneider). In this manual the concept is not limited to illnesses with a deteriorating course, although a minimal duration of illness is required since the accumulated evidence suggests that illnesses of briefer duration (here called Schizophreniform Disorder) are likely to have different external correlates, such as family history and likelihood of recurrence. The approach taken here excludes illnesses without overt psychotic features, which have been referred to as Latent, Borderline, or Simple Schizophrenia.† Such cases are likely to be diagnosed in this manual as having a Personality Disorder such as Schizotypal Personality Disorder. Illnesses with onset after mid-adult life are also excluded, and may be classified as Atypical Psychosis. Furthermore, individuals who develop a depressive or manic syndrome for an extended period relative to the duration of certain psychotic features or before the psychotic features appear, are not classified as having Schizophrenia but rather as having either an Affective or Schizoaffective Disorder. Thus, this manual utilizes clinical criteria that include both a minimum duration and a characteristic symptom picture to identify a group of conditions that has validity in terms of differential response to somatic therapy; presence of a familial pattern; and a tendency toward onset in early adult life, recurrence and deterioration in social and occupational functioning.

Deterioration from a previous level of functioning. Schizophrenia always involves deterioration from a previous level of functioning during some phase of the illness in such areas as work, social relations, and self-care. Family and friends often observe that the person is "not the same."

* Although Schizophrenia is most likely a group of disorders of differing etiologies, common usage refers to "Schizophrenia" rather than the technically more accurate term, Schizophrenic Disorders.

† The clinician wishing to use these non-DSM-III diagnoses may do so since they are included in ICD-9-CM (p.399).

Characteristic symptoms involving multiple psychological processes. Invariably there are characteristic disturbances in several of the following areas: content and form of thought, perception, affect, sense of self, volition, relationship to the external world, and psychomotor behavior. It should be noted that no single feature is invariably present or seen only in Schizophrenia.

Content of thought. The major disturbance in the content of thought involves delusions that are often multiple, fragmented, or bizarre (i.e., patently absurd, with no possible basis in fact). Simple persecutory delusions involving the belief that others are spying on, spreading false rumors about, or planning harm to the individual are common. Delusions of reference, in which events, objects, or other people are given particular and unusual significance, usually of a negative or pejorative nature, are also common. For example, the individual may be convinced that a television commentator is mocking him.

Certain delusions are far more common in this disorder than in other psychotic disorders. These include, for instance, the belief or experience that one's thoughts, as they occur, are broadcast from one's head to the external world so that others can hear them (thought broadcasting); that thoughts that are not one's own are inserted into one's mind (thought insertion); that thoughts have been removed from one's head (thought withdrawal); or that one's feelings, impulses, thoughts, or actions are not one's own but are imposed by some external force (delusions of being controlled). Less commonly, somatic, grandiose, religious, and nihilistic delusions are seen. Overvalued ideas may occur (e.g., preoccupation with the special significance of certain dietary habits), or markedly illogical thinking (e.g., thinking that contains clear internal contradictions or in which conclusions are reached that are clearly erroneous, given the initial premises).

Form of thought. A disturbance in the form of thought is often present. This has been referred to as "formal thought disorder," and is distinguished from a disorder in the content of thought. The most common example of this is loosening of associations, in which ideas shift from one subject to another completely unrelated or only obliquely related subject, without the speaker showing any awareness that the topics are unconnected. Statements that lack a meaningful relationship may be juxtaposed, or the individual may shift idiosyncratically from one frame of reference to another. When loosening of associations is severe, incoherence may occur, that is, speech may become incomprehensible. There may also be poverty of content of speech, in which speech is adequate in amount but conveys little information because it is vague, overly abstract or overly concrete, repetitive, or stereotyped. The listener can recognize this disturbance by noting that little if any information has been conveyed although the individual has spoken at some length. Less common disturbances include neologisms, perseveration, clanging, and blocking.

Perception. The major disturbances in perception are various forms of hallucination. Although these occur in all modalities, by far the most common are auditory, frequently involving voices the individual perceives as coming